

****DISCHARGE SUMMARY****

****PATIENT DEMOGRAPHICS****

Name: Patient 45

Age: 62 years old | Sex: Male

MRN: 000-TEST-45

Admission Date: 06/12/2026 | Discharge Date: 06/15/2026

Attending Physician: Dr. M. Richardson, MD

PRIMARY DIAGNOSIS & COMORBIDITIES

Patient 45 presented with acute abdomen and was found to have acute cholecystitis with cholelithiasis and concurrent incarcerated umbilical hernia. Past medical history significant for COPD (Gold Stage II, on home O₂ at 2L/min nocturnal) and peripheral arterial disease (PAD) affecting bilateral lower extremities with claudication at 50 yards. Social history: former smoker, quit 8 years ago, lives with spouse.

HOSPITAL COURSE

Pt admitted via ED on 06/12 with RUQ pain, fever 38.2°C, and leukocytosis (WBC 14.8K). Abdominal imaging (ultrasound + CT with contrast) confirmed acute calculus cholecystitis without perforation, plus irreducible umbilical hernia. Patient counseled on surgical options; elected for combined laparoscopic cholecystectomy and hernia repair.

Underwent surgery 06/13/2026 under general anesthesia with epidural anesthesia adjunct. Laparoscopic approach initiated with 3 trocar sites: 10mm umbilical (converted fascial closure post-hernia repair), 5mm epigastric, 5mm midaxillary. Cholecystectomy performed without

incident; gallbladder grossly edematous with multiple stones, no bile duct stones identified on intraoperative cholangiogram. Mesh-reinforced umbilical hernia repair completed laparoscopically using lightweight composite mesh with fascial approximation. No conversion to open procedure required. EBL minimal (~25 mL). Operative time 84 minutes. Patient tolerated procedure well without complications.

Post-op course uncomplicated. SpO₂ maintained >94% on home oxygen regimen. Pain controlled with multimodal analgesia (see medications). Trocar sites dressed with gauze; umbilical site reinforced given hernia repair. Bowel function returned POD#2 (flatus, soft BM). Diet advanced: clear liquids POD#1 → full liquids POD#2 → regular diet POD#3 without nausea/vomiting. Discharged POD#3 in good condition.

MEDICATIONS AT DISCHARGE

Medication	Dose	Frequency	Duration
Acetaminophen	650 mg	q.6h. PRN pain	2 weeks
Ibuprofen	400 mg	BID w/ food	10 days
Omeprazole	20 mg	q.d.	4 weeks
Colace (docusate sodium)	100 mg	BID	Until bowel regularity
Albuterol inhaler	2 puffs	q.4-6h. PRN SOB	Ongoing
Tiotropium (Spiriva)	18 mcg	q.d. inhaled	Ongoing COPD maint.

WOUND CARE & ACTIVITY

****Trocar Site Management:****

Keep dressings dry × 7 days. May shower after dressings removed; avoid tub baths × 3 weeks. Clean sites daily with mild soap & water; report signs of infection (erythema, purulent drainage, warmth).

****Activity Restrictions:****

No heavy lifting or straining >10 lbs × 6 weeks. No driving while on o